

SPMM
Smart
Revise

Forensic Psychiatry I

Paper B

Syllabic content 12.1

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1. Relationship between crime and mental disorder

- ★ Most people with a psychiatric disorder are not violent. The statistical significance of the association of MD with violence is robust but there are difficulties in conducting the research, such as ascertaining the impact of confounders and mediators.
- ★ Various approaches to studying mental illness and violence include estimating the prevalence of:
 - Violent acts in those with mental illness;
 - Mental illness in individuals who have committed violent acts;
 - Prevalence of violence in the community regardless of mental health (community-based epidemiological studies). (Walsh 2002)

Prevalence of crime among the mentally disordered:

- ★ Violence is 4 to 6 times more common in the mentally ill than the general population. There are much higher rates of arrest in those with a mental disorder than the general population. But mental illness can be an attributable risk factor in less than 10% of all crimes.
- ★ Personality disorder, alcohol and drug misuse are all much higher risk factors for violence than mental illness. Factors such as male gender, age between 15 and 30 years, being socioeconomically deprived and past history of violence are much greater risk factors than schizophrenia or mental illness.
- ★ However, individuals with a mental disorder, are much more likely to be victimised, be involved suicide and self-harm.
- ★ Monahan & Applebaum (2000, MacArthur Risk Assessment Study) estimated the prevalence of community violence in discharged patients. 9% of schizophrenia patients were violent in the first 20 weeks after discharge. This compares with a violence prevalence of 19% for depression, 15% for bipolar disorder, 17.2% for other psychotic disorders, 29% for substance misuse disorders and 25% for personality disorder alone.
- ★ 10% in community with schizophrenia committed a violent act in a 12 months period, compared to 2% without a mental disorder (five times higher). Comorbidity with substance abuse increased this percentage to 30%. The five-times elevated risk also persists when conviction rates are considered. Men with schizophrenia are up to five times more likely to be convicted of serious violence than the general population (Wallace *et al.*, 1998). Note that 99.97% of those with schizophrenia would not be convicted of serious violence in any given year.
- ★ The probability that any given patient with schizophrenia will commit homicide is approximately annual risk of 1:3000 for men and 1:33 000 for women.

Prevalence of mental disorders in offenders:

- ★ Fazel and Danesh (2002) systematically reviewed 62 surveys of prison studies covering 12 countries and around 22,790 prisoners and found amongst men, rates of psychosis was 3.7%, major depression was 10% and personality disorder was 65%. 1% of sentenced prisoners suffer from an organic mental disorder.
- ★ Shaw (2006) determined 34% of homicides are related to a mental disorder and 5% of homicides are linked to schizophrenia; 10% of homicide offenders have active symptoms at the time of the offence.
- ★ Brugha et al. (2005) suggested that psychosis is ten times commoner in prison than in the general population.
- ★ Mental illness (especially psychosis), is more common in the remanded population who were violent vs. non-violent (Hodgins et al., 2004)

Why is mental disorder common in the criminal justice system?

- ★ Epidemiological association with violent offending (Mullen, 2006)
- ★ Prison psychosis: Incarceration is a 'causal' agent attributed to mental disorders such as psychosis. Although now disputed, some face validity is present for such an argument.
- ★ Institutionalisation bias: Seddon (2007) suggested, following Foucault, that prison is a method of confining undesirable elements of society, for example, mental health patients, as well as criminals.
- ★ Penrose Law: Lionel Penrose examined the association between the number of individuals in mental institutions and crime rates across Europe. According to him, the occupancy rates in mental 'institutions' increased with reduction in the number of murders, the number of live births per 1000 population and most importantly, the number of individuals in prison (Gunn, 2000). The inverse association with prison occupancy has been confirmed by many other studies conducted later. This observation of inverse relationship between prison occupancy and mental institution occupancy rates is often termed as Penrose's Law (Kelly, 2007)

2. Specific Crimes

Homicide

- ★ This is defined as the deliberate act of killing another individual unlawfully. Homicide is comparatively rare occurring in 1.2/100,000 in the UK (UNODC, 2012)
- ★ Between 1980-2004 – number of homicides committed by mentally disordered offenders (MDO) fell from 120 per year to 20 per year; it also fell as a proportion of all homicides. Over the past few years, it has been approximately 50 a year (NCI 2013).
- ★ 10% of individuals convicted of homicide have some form of abnormal mental state at the time of offence – of which 2/3^{rds} have psychoses. Notably, 9% of homicide offenders had

schizophrenia, 12% other psychotic disorders, 54% secondary PD diagnoses (Swedish studies).

- ★ 10% of homicide MDOs have had contact with mental health services within the past year (c.f. suicide – which has a much higher rate)
- ★ The annual risk of a person with schizophrenia committing homicide in the UK is 1 in 10,000 but that of acquiring a conviction for violence is 1 in 150 (Wallace et al. 2004). The annual homicide rate in the UK is approximately 1 in 100,000 so even a tenfold increase in risk in schizophrenia will not perceptibly affect the clinical risk when individual cases are considered.
- ★ Victims of homicides by MDOs are more often acquaintances (esp. a member of the perpetrator's family), rather than strangers.

Filicide

- ★ Filicide refers to the act of killing one's own child by the mother. D'Orban (1979) described 6 types of filicides (see the table below). There are also other accepted sub-classifications – *altruistic, psychotic, accidental, unwanted and spousal revenge related*.

Type	Description
Battering mothers	Impulsive killing (loss of temper)
Mentally ill mothers	Most commonly depression (~100% in some series of cases; Marleau et al., 1995; Stanton et al., 2000); psychotic illness and/or personality disorders and dissociative responses. Borderline personality disorder (5) or dependent personality disorder (4) were the commonest types of axis II disorders. McKee and Shea (1998) reported a prevalence of psychosis in 40% and major depression as 25%.
Neonaticides	Neonaticide is not defined by the Infanticide Act, but in medical publications usually refers to the killing of a child during the first 24 hours of life and such cases are often unknown to healthcare services.
Retaliating women	Aggression towards the spouse is displaced on to the child.
Unwanted children	Passive neglect or active aggression associated with unplanned pregnancies and socioeconomic hardships
Mercy killing	Victim is truly suffering; no other secondary gain for the mother

Infanticide is defined as the intentional killing of an infant by the mother. Infanticide Act 1938 (in the UK) employs the term infanticide when a mother '*causes death of her child under the age of 12*

months by wilful act or omission, but at the time of the act or omission the balance of her mind was disturbed by reason of her not having fully recovered from the effect of her having given birth to the child or by reasons of the effect of lactation consequent on the birth of the child'. (Retrieved from <http://jrs.sagepub.com/content/97/2/57.full>)

Official figures, estimate the incidence of infant homicide in Britain to be between 30 and 45 per year (Marks and Kumar, 1993). Others consider the official figures as an underestimate (Resnick, 1970). Important risk factors that should be picked up in the antenatal history are substance abuse and mental illness. *There is some evidence that covert video surveillance may be useful to investigate 'high risk' cases to prevent such outcomes (Southall et al. 1997)*

Infanticide is considered as a psychiatric defence (see Section on Psychiatric Defences below).

Other offences

Murder-suicide : It is defined as a murder that is followed by the suicide of the perpetrator within one week of the homicide. The incidence is 0.2–0.3/100,000 per year (Eliason, 2009). Depression is more common in murder-suicides (20-60%) than among homicide perpetrators, but substance misuse and criminal history are less frequent in murder-suicides.

There are some clinical presentations that show an association with murder-suicides:

- A middle-aged man who is recently separated or facing imminent separation from his intimate partner and is depressed and has access to firearms;
- An older male who is the primary caregiver for a spouse who is ill or debilitated, where there is a recent onset of new illness in the male, depression, and access to firearms.

Parricide : The act of killing one's parents (mostly patricide – killing the father ; sometimes matricide – killing the mother). It is rare and amounts to 2% of all homicides.

- In most cases, the perpetrator is a son, aged between 12 and 50, motivated by revenge for sexual abuse or financial interests (Seagrave 2009).
- Usually, a spontaneous act occurring during a domestic argument (Shon & Roberts 2010), because of resentment, explosive anger, jealousy, and general family discord. Often occurs in a background of chronic intrafamilial conflicts and repeated homicidal or suicidal threats. Sudden mood changes in family members, such as signs of irritability, occurring in such a background family dynamics are clinically noticeable risk factors

Non-fatal assaults

There are 1 million cases a year (2033/100,000) in England & Wales. This can be classified as

- *Common assault* – intentionally/recklessly causes another person to apprehend the application of immediate unlawful force (the assault is the apprehension of such contact.)
- *Aggravated assault* – when the individual attempts to cause serious bodily injury to another or causes such injury purposely, knowingly, or recklessly under circumstances

manifesting extreme indifference to the value of human life; or attempts to cause or purposely or knowingly causes bodily injury to another with a deadly weapon.

- *Battery* – if he/she intentionally/recklessly applies unlawful force to the body of another person. (A battery is committed when the threatened force actually results in contact with the other and that contact was caused either intentionally or recklessly.)

Approximately 9% of individuals convicted of non-fatal violence have been found to have schizophrenia and the majority of the cases are under the influence of alcohol.

Shoplifting:

- ★ Only 3.2% of the cases involved mentally ill patients
- ★ There is a close link between shoplifting and affective disorders, alcoholism and drug addiction. Depression is especially associated if the shoplifter is a middle-aged woman (24-30% middle aged female shoplifters may be depressed: Gibbens, 1971).
- ★ Shoplifting generally peaks in adolescence.
- ★ Most offenders do not reoffend if caught.
- ★ **Kleptomania** is an impulse control disorder. It is classified under ICD-10 F63 'Habit and impulse disorders'. In this case, the theft is usually petty and impulsive, done without the help of others or planning. Around 1-2% of shoplifters have a history consistent with kleptomania. Most kleptomaniacs are women. But the severity of kleptomania is not biased by gender. Nearly 50% of kleptomaniacs have a comorbid personality disorder. The most common PD is paranoid personality followed by histrionic personality.

Arson:

- ★ 1 in 4 of fire setters *intentionally* started the fire; this offence is known as arson. Clearly, fire setting can also be accidental (such as falling asleep with a cigarette), and such incidences can occur in those with a mental disorder.
- ★ It is often hard to distinguish mental disorder-related arson from a non- mental disorder related arson in terms of clinical history. There is rarely a strong causal relationship between mental illness and arson, and not guilty by the reason of insanity is only used as a defence for rare cases of delusional behaviours.
- ★ Among psychiatric patients, Geller and Bertsch (1985) found that 26% of the patients had a history of fire-setting behaviour, and 16% of these patients had actually set fires.
- ★ Psychiatric disorders showing an association with fire-setting include personality disorders, psychoses (8% of firesetters with mental illness had schizophrenia, 11% bipolar disorder), learning disability and intoxication states (Rix 1994). Alcohol misuse very common; Low IQ is over-represented.

- ★ Fire-raisers appear to be mostly young adult males; many have considerable relationship difficulties. They are often unmarried, poorly educated, isolated, and unemployed or unskilled labourers.
- ★ Arson is more common in males (2.5 times more in males) though the proportion of female arsonists is increasing. When women commit repeated acts of fire-raising, they are more likely than their male counterparts to be given a psychiatric disposal.

Primary gain	Secondary gain	Other motives
Sense of relief (from boredom or tension) or excitement (including sexual pleasure)	Financial (e.g. insurance claim, housing)	Accidents
Delusional motive	Extortion	Carelessness
Revenge	To conceal or facilitate a crime	Curiosity (esp. in children)
Jealousy	Vandalism	
Self-destruction/suicide	Political protest	
Heroism/recognition		
Seek attention		

Adapted from Burton et al. J Am Acad Psychiatry Law 40:355–65, 2012

- ★ Faulk proposed two broad groups:
 - Group I: Cases in which the fire served as a means to an end (e.g. revenge, fraud, or a plea for help) (Instrumental fires)
 - Group II: Cases where the fire itself was the phenomenon of interest (e.g. pyromania). Some pathological fire setters (occurs on 2+ occasions – display preoccupation with fire though no excitement, often no motive).
- ★ Pyromania (occurs on 2+ occasions, and results in relief of tension): **Pyromaniacs** are fire-raisers who derive a pathological excitement from setting the fire, attending the scene, busying themselves with it, or having called out the fire brigade in the first instance. DSM-IV lists six diagnostic criteria - Deliberate and purposeful fire setting on more than one occasion; Affective arousal and tension prior to the act; Fascination with, and attraction to, fire and its situational context; Pleasure, gratification or relief when setting fires or witnessing or involvement in their aftermath; The exclusion of other causes (see above); The fire-setting is not 'better accounted for' by conduct disorder, or antisocial personality disorder.
- ★ Juvenile fire setters, who are often >50% of all arrests –those younger than 10 years old- the motive is often one of curiosity. It is commoner in females than males, detained in secure hospitals. In females – there is often an *emotional meaning* regarding property and others (communicative or expressive act) associated with revenge/jealousy and hatred (displaced aggression).
- ★ Reoffence: Recidivism rates vary from 4% - 20%. The presence of a mental disorder is a poor prognostic feature (11% reconviction in MDOs vs. 4% in non-MDOs) associated with higher

reoffence rates. Main factors for recidivism in MDOs include: childhood firesetting problems, younger age at first firesetting and arson, total number of firesetting offences, no concurrent charges other than arson, verbalized threats to commit arson, setting fires alone, unmarried and low IQ.

- ★ Treatments for fire setting are largely behavioural or focused on intervening in the family or intrapersonal stresses that may precipitate episodes of fire setting. Behavioural treatments like aversive therapy have helped fire setters (Koles and Jenson, 1985). Other treatment methods rely on positive reinforcement with threats of punishment and stimulus satiation. No established role has been shown for psychodynamic therapy.

Stalking

- ★ Stalking is behaviour characterised by repeated intrusions involving unwanted contacts, including following, approaches and surveillance and unwanted communications including telephone calls, emails, letter and graffiti
- ★ **Behaviours associated with stalking are** Threats, assaults, ordering or cancelling goods or services on victims behalf, sending unsolicited gifts, initialising spurious legal actions and making complaints.
- ★ The prevalence is surprisingly high in modern society. Mullen interviewed 6300 females, 15% were stalked sometime in their life. 80-90% of stalkers are men and 80-90% of the victims are women.
- ★ Victim profile: Most common victims are women in their reproductive years who have had a sexual relationship with the stalker.
- ★ Perpetrator profile: Stalkers are often unemployed or underemployed at the time of stalking and better educated than other criminals. The most common form of stalking involves men stalking women with whom they had been sexually intimate (Melow, 1988).

Typology of stalking perpetrators (Mullen 1999)

- The rejected who pursues ex-intimates, either in the hope of reconciliation or for vengeance.
- Intimacy seekers who stalk someone they believe they love and they think they will reciprocate
- Incompetent suitors, who inappropriately intrude someone, usually seeking a date or brief sexual encounter
- The resentful who pursue victims to exact revenge for actual or perceived injury.
- The predatory whose stalking forms a part of sexual offending.
- ★ When the psychiatric status is ascertained in stalkers, the primary diagnosis is often a personality disorder. Cluster B personality disorders of the borderline, narcissistic and histrionic types are predominant in ex-intimate stalkers. Substance abuse is common.

Delusional disorders (most commonly erotomanic, followed by jealousy and persecution), schizophrenia, affective and organic psychosis are common in stranger and star stalkers. Mental illness is not a notable factor in those who stalk or attempt to assassinate public figures (Fein and Vossekuil 1998)

- ★ **Effect on victims:** Serious disturbances in emotional, social, interpersonal and work functioning. PTSD has been reported in 37-60% of the victims. Depression, anxiety, insomnia, somatisation disorders and substance misuse are all reported. Victims may move residence, change jobs or stop working while some may even harm themselves.
- ★ **Risk Assessment:** Stalking is a common behaviour driven by various motives. There is no standardised risk assessment tool. Factors likely to increase risk of assault in stalking (Palarea 1999, Mullen 2000) include
 - Substance misuse
 - History of offending behaviour
 - Male gender
 - Making threats of violence or suicide or fantasising about assaults
 - Presence of PD (Narcissistic or antisocial)
 - Unemployed and socially isolated
 - Access to victims
 - Sense of desperation (crisis periods)
 - History of non-compliance to treatment
- ★ **RCPsych Stalking survey:** In 2007, the College commissioned a study of stalking of its members. Nearly 22% psychiatrists considered themselves to have been stalked. Approximately 1 in 3 had experiences of stalking behaviours that met the legal definition of harassment. On average, one College member became the victim of a new series of stalking behaviours every week (51 new members per year).
- ★ Stalkers who target mental health professionals are usually males with a prior history of stalking, with an established axis-1 or 2 disorder under the care of the stalked clinician. Many such stalkers are motivated by a desire for a greater intimacy.
- ★ **Management:** Clinical management is based on
 - The nature of the contributory mental disorder and an understanding of what is sustaining the behaviour
 - Confronting self-deception, which minimise or justify the behaviour
 - Instilling empathy for the victims plight
 - Addressing inadequate or inappropriate social and interpersonal skills
 - Combating substance misuse.
- ★ Prosecutions are under the 'Protection from Harassment Act 1997' (England & Wales). Sometimes restraining orders may lead to violence or increase in the intensity of stalking.

Sexual offence

- ★ There are various types of offending in this group– frotteurism, fetishism, exhibitionism, paedophilia, sadomasochism, voyeurism amongst others.
- ★ Most sex offenders do not have a major mental illness (Grubin & Gunn, 1991). However, people with a mental disorder may show abnormal sexual behaviour. The presence of mental disorders including learning disability or personality disorder, schizophrenia, alcohol use and hypomania is related to disinhibition. Notably, only <10% of sex offenders – have a mental illness. There is a similar psychosexual profile between MD and non-MD offenders.
- ★ Sexual offences constitute 1% of all offences: this figure is most likely to be an underrepresentation of actual prevalence. Almost exclusively, men are the perpetrators.
- ★ The offenders are widely heterogeneous and no consistent or absolute correlation is seen between a sexual offence and a sexual disorder. Most offenders have more than one category of deviant sexual behaviour.

Specific sexual offences

- ★ **Paedophilia and CSA (child sexual abuse):** CSA is the involvement of children in sexual activities, which they do not fully comprehend and to which they cannot give informed consent.
 - 80% of offenders against children are either relative (13%) or are known to them (68%). The offender is usually a male (90%) and commonly a relative. One-third of the offenders are adolescents. Most victims are known to the offender and up to half are related. Girls are twice likely to be the victims.
 - Among perpetrators of child sexual abuse, characteristics of high deviancy group as found by the STEP project commissioned by the Home Office were: individuals committing offences outside and inside the family; individuals offending against both boys and girls; high deviants were twice as likely to have committed previous sexual offences; more likely to have been abused as a child.
 - Fifty to 70% of paedophiles can be diagnosed as having another paraphilia, such as frotteurism, exhibitionism, voyeurism, or sadism. Paedophiles are approximately 2.5 times more likely to engage in physical contact with a child than simply voyeuristic or exhibitionistic activities.
- ★ **Incest:** By definition, penetration must have happened in order to use the term incest for an offence. There is a decline noted in reported incestual crimes. Sexual relations with step-relatives are not classified as incest in England, but Scotland includes this as incest. 30% reported incest is against children.

- ★ **Indecent exposure:** This may be due to exhibitionism or secondary to alcohol, drug use or rarely psychiatric problems such as dementia, hypomania. The offender may or may not masturbate after exhibiting; he rarely communicates with the victim. The offender usually stops reoffending after a conviction; serious progressive crimes are very rare. Nearly 20—30% reoffend and if such reoffence leads to a conviction, the prognosis is very poor in terms of high recidivism. Indecent exposure is often an impulsive act associated with a decreased satisfaction in life. Most offenders commit such an offence at times of stress and are more likely to be married and not show characteristics of other sexual offenders. An inhibited guilty exhibitionist (flaccid penis) is less risky than an angry, aggressive exhibitionist (erect when offending).
- ★ **Internet based sex-offending:** This is often associated with emotional dysregulation and intimacy deficits and depressive and negative mood states. Often, individuals have no prior convictions. Most do not reoffend unless there is a past history of other forms of sexual offending. Typology: (1) seeking emotional satisfaction – normally distressed, seek solace through the internet (2) Intimacy deficit – socially isolated people (3) Hypersexuality – for high levels of sexual behaviours

Management:

- ★ A number of psychological characteristics have been associated with sexual offending. These dynamic risk factors are often considered when assessing an offender (Thornton, 2002):
 - Sexual interests (including sexual preoccupation, as well as sexual preference for children)
 - Distorted attitudes and beliefs (cognitive distortions, and beliefs supportive of rape)
 - Socio-affective management (for instance, emotional regulation and intimacy difficulties)
 - Self-management (for instance, poor problem-solving abilities, lifestyle impulsiveness).
- ★ Penile plethysmography: As sex offenders often deny their intent, self-reports are unreliable. Penile plethysmography allows to determine the level of sexual arousal by measuring changes in penile tumescence when visual (images) or auditory cues (stories) of deviant sexual materials are presented (Barker & Howell, 1992).
- ★ CBT: The work of Finkelhor has been influential in the development of CBT. His 4 stage model of sexual offending includes 1. Motivation to abuse sexually 2. Overcoming internal inhibitions 3. Overcoming external inhibition and 4. Overcoming the resistance of the victim. Sex offenders will commit an offence only after they have moved through the four stages.
- ★ Treatment strategies involve: Understanding the offence cycle, Challenging distorted thinking, Understanding the harm done to victims, Fantasy modification, Social skills and anger control and Relapse prevention work
- ★ Pharmacotherapy: In the minority of sex offenders who have a contributing mental illness, the treatment of the underlying mental illness could considerably reduce the risk of further sex offending.

- Cyproterone acetate acts mainly by blocking testosterone receptors. The main uses of this drug are the reduction of sexual drive and, in higher dosage, the treatment of prostatic carcinoma (Bradford, 1985). Side effects include deranged liver functions, osteoporosis gynaecomastia. Cyproterone can cause deterioration of any depressive tendency – it is not be used if there is a history of severe depression in the past.
- Medroxyprogesterone acetate is the main anti libidinal preparation used in the USA. It induces the hepatic enzyme testosterone alpha-reductase, thus enhancing the metabolic clearance of testosterone and reducing circulating testosterone levels.
- An initial report suggested successful use of buspirone in the treatment of a patient with transvestic fetishism (Fedoroff, 1988); this was followed by a number of reports highlighting the potential role of SSRIs and other antidepressants in treating paraphilia.

Rates of reoffence

- ★ 'The key question in measuring recidivism is whether the once-convicted offender commits a second sexual offence when returned to the community' (Becker, 1994). "Studies suggest that less than one in five of a general sample of sex offenders released from prison go on to commit a further sex offence"(Grubin & Wingate, 1996)
- ★ **Abel study:** Between 1977 and 1985, Abel et al. interviewed a large sample (n = 561) of nonincarcerated male sex offenders under a US federal certificate of confidentiality. This is the largest study looking into the non-imprisoned sex offenders. Surprisingly, nearly 30% were married; most were employed; 40% had at least one year of college education; 67% targeted only females, 12% targeted only males, and 21% targeted both. While 56% engaged only in nonincestuous behaviour (molesting or assaulting nonrelatives) and 12% engaged only in incestuous behaviour, 23% engaged in both incestuous and nonincestuous behaviour. The subjects molested young boys 5 times more often than young girls. Most had multiple paraphilias. In more than 50% subjects, the paraphilic tendencies started before age 18.
- ★ In general, 75-80% of sexual offenders have no previous conviction for a sexual offence, indicating that most convictions are of 'first-time' offenders. Of those who had at least one prior sexual conviction, nearly 40% had more than one. 71% of those whose first sexual conviction was for an offence against a child, 71% repeated their sexual offences against children only.
- ★ The proportion reconvicted of any sexual offences during 6 years follow-up was relatively low: less than 10% (rapists 19%, CSA 13% Hanson et al., 2009). However, those who were reconvicted committed very serious crimes. Reoffence rates increase with longer duration of observation.
- ★ The proportion reconvicted varied, according to the type of victim at index offence. Lowest reconviction: Index offence against a child in the own family. Incest offenders have the lowest untreated recidivism rates, often less than 10%. Highest reconviction: Index offence against a

child outside one's family. Among these those who molest boys generally have higher reoffense rates (13% to 40%) than those who molest girls (10% to 29%). *Multiple paraphilias* can also be a powerful indicator of recidivism. Among those who offend sexually against adults, if the index offence is against a stranger rather than known adult, then reconviction rate was higher.

- ★ Rapists are criminally more versatile than child molesters and are more likely to reoffend non-sexually than sexually. Among child molesters, the risk of sexual recidivism is higher for homosexual and extra-familial sex offenders than for heterosexual and incest offenders.
- ★ Predictors of a sexual offence (Lievore 2004): Single variables are clearly limited in their predictive ability of offending. But some of these may be
 - Sexual deviancy: Higher number of sex offences, especially more than one type of sexual offence, and previous criminal history.
 - Diversity in offending, including violent and general crimes;
 - Offences against male children carries more reoffending risk than offences against girls; there is a lower risk of recidivism if the primary offence is an incest
 - Phallometric evidence of response to paedophile stimuli and/or nonsexual violence
 - Elevated score on Hare Psychopathy Scale (Rice *et al.*, 1990)
 - At time of index offence, reduced self-esteem, impaired victim empathy (high risk) or increased anger
 - Being a victim of childhood sexual abuse,
 - Presence of violent sexual fantasies
 - Longstanding social isolation (present in some sexual murderers)
 - Choice of occupational location to facilitate access to potential victims
 - Use of sadomasochistic or paedophilic pornography
 - Non-compliance or failure to complete treatment. This is often associated with dissocial traits.
 - Often more deviant sexual practices are seen as relatively younger victims.
 - Psychological maladjustment, including substance use or abuse, antisocial attitudes and personality disorders

Domestic violence in the UK

- ★ One-third of women survivors (31 percent) of domestic violence do not tell anyone about it (40% of raped and 9% of stalked women do not disclose to anyone). Men were less likely to tell than women.
- ★ Friends, neighbours and relatives of the victims were by far the most likely to hear of incidents Of the 47% of all victims who had told someone this involved disclosing to a

friend or relative. The police were the next most likely to hear of the incidents (20%) followed by medical staff (10%). But medical professionals offer advice or support more often than the police.

- ★ The police come to know about less than one in four cases (17 to 23 percent for women) of domestic violence and less than one in seven cases of sexual assault. This amounts to a meagre total of 12% of individual assaults that get picked up using surveys. The police often come to know about incidents from someone other than the victim.
- ★ One-in-eight women suffering domestic violence thought that the police would make matters worse rather than better.
- ★ Surveys which include sexual violence and a broader range of abuse indicate that domestic violence affects 1 in 4 women at some time in their lives. Women most at risk of domestic violence are younger women below the age of 40yrs (44% of all violent attacks are the result of domestic violence).

2. Relationship between specific mental disorders and crime

Schizophrenia and Psychoses

- ★ Fazel & Grann (2006) found a population attributable risk fraction of schizophrenia in causing violence as just 2.3%, which increased to 5% if psychosis is looked at broadly. This attributable risk fraction was higher in women than men across all age bands. The attributable risk fractions were lowest in those ages 15–24 (2.5%).
- ★ In countries with more liberal gun laws, the risk attributable to psychosis is lower for homicide, but some argue that those with schizophrenia are responsible for 5–10% of homicides irrespective of the baseline homicide rate (Wallace et al., 2004). Wallace *et al.* (2004) estimated that 6–11% of all violent convictions are attributable to schizophrenia.
- ★ There is an increasing support for a two-type model of violence in schizophrenia (Steinart et al. 1998):

Type 1 violence in schizophrenia	Type 2 violence in schizophrenia
People displaying type 1 violence have organised delusional systems related to the violence, do not have prominent history of conduct problems as a child or adult and usually commit violence after entering treatment, almost always attacking a carer or acquaintance. It is possible that among homicide offenders type 1 violence is overrepresented.	Those with type 2 violence tend to have disorganised clinical syndromes, history of conduct disorder, early onset substance misuse, and usual a diverse set of offending prior to diagnosis and commit domestic and non-domestic violence. Most violence in the schizophrenia population is attributable to type 2.

- ★ **Threat/control override:** A specific threat/control override syndrome was proposed by Link & Steuve to explain the relationship between violence and psychosis. The threat/control override symptoms represent experiences of patients feeling that people are trying to harm them (threat) and experiences of their minds being dominated by forces outside their control (control override). This was based on retrospective and rather a weak methodology. But the MacArthur Violence Risk Assessment Study has cast doubt on the importance of threat/control override delusions as mediators for violence (Appelbaum *et al.*, 2000). Neither delusions in general nor threat/control override delusions, in particular were found to be associated with an increased risk of violence.

Alcohol and drug misuse

- ★ Substance use is fairly common in mentally disordered offenders. Studies indicate that >50 % of prison inmates misuse substances while >40% misuse alcohol, when compared to 2% and 4% respectively in the general population.
- ★ Acute intoxication is associated with disinhibition and disorganisation. It is associated with nearly 50% of non-fatal assaults; 66% of murders.
- ★ Alcohol use hugely inflates the risk of violence among mentally disordered offenders. For example, patients with schizophrenia who also misuse substances are 25 times more likely to be at risk of violent offending than the general population, compared to the risk ratio of 3.6 in those who have schizophrenia but do not have comorbid substance misuse.

Personality Disorder

- ★ Offenders with PD mostly offend against known victims (including healthcare professionals)
- ★ Women with PD are more likely to offend against children than women offenders without PD.
- ★ **Any PD:** 78% remanded men; 64% sentenced men and 50% female prisoners and 65% of forensic patients have a diagnosable personality disorder.
- ★ **Antisocial PD:** 63% remanded men, 49% sentenced men and 31% female prisoners and 67% of forensic patients have antisocial personality disorder. Among individuals with antisocial personality disorder, there is a tenfold increase in the risk of violence compared to the general population.
- ★ **Borderline PD:** 23% remanded men, 14% sentenced men and 20% female prisoners and 25-60% of forensic patients have borderline personality disorder.
- ★ **Narcissistic PD:** 8% remanded men, 7% sentenced men, 6% of female prisoners; 25% of forensic patients
- ★ **Paranoid PD:** 29% remanded men, 20% sentenced men, 16% of female prisoners; 18% of forensic patients
- ★ **Psychopathy** is seen in 15-30% of the forensic population in contrast to 4% of the general population. In Hare's Psychopathy Checklist (Revised), a score >25 indicates psychopathy and increased the risk of violence.
- ★ There are two clusters of features seen in psychopathy:
 - **Factor 1 – callous-unemotional** – arrogant interpersonal style, grandiose, glib, deceitful, lacking empathy lacks remorse, shallow and labile, deficient affective experience. These traits are more influenced by biological makeup and heritability.
 - **Factor 2 – antisocial** – impulsive, sensation-seeking, lack of planning, irresponsible, parasitic lifestyle.

Learning disability:

- ★ **IQ:** It is possible that duller offenders get caught and convicted more often than the brighter ones, over representing learning disabled in crime. But non-convicted self-reported offences are also more in low IQ group. Poor abstract concepts may lead to recurrent law breaking.
- ★ Studies by West show that IQ is one of the 5 factors associated with the development of offending behaviour. Studies have shown a consistent statistical association between low IQ and offending. 20% of those with IQ below 90 become delinquent compared with 9% of the IQ between 91-98 and 2% above IQ 110. Lyall found that 2% of 358 adults with LD, living in residential homes or attending day services had contact with police in a year. But there was a marked tendency to under report offences. Harris found that the overall prevalence of aggressive behaviour amongst people with LD was 17.6% (day hospital 9.7% and inpatient 38.2%).
- ★ Different studies report prevalence rates of intellectual disability within prison populations varying from around 2% to around 40%
- ★ Reviews of high-security provision indicate that offenders with intellectual disability have the longest duration of stay and are the most difficult to discharge because of the lack of availability of suitable discharge resources (hebw.cf.ac.uk)
- ★ It is generally asserted that people with LD are more likely to commit sex or fire-raising offences with property offences being the most common (Hodgins).
- ★ Offending is more likely in mild and moderate than severe LD and their offences may require some planning and complex skills.
- ★ **Sex Offences:** Sex offenders with a learning disability have a greater tendency to offend against younger male children. Using more than 4000 psychiatric reports, Hawk found “rate of sex offence charges was nearly twice as high among mentally retarded as among defendants who were not retarded”. Sex offenders with intellectual disability may have a high incidence of family psychopathology, psychiatric illness, social naivety, poor ability to form normal sexual and personal relationships, poor impulse control and low self-esteem. Management is by group SOTP and pharmacotherapy (SSRI and anti libidinal)
- ★ **Reoffence rates:** Studies have found re-offending rates of untreated offenders of between 40 and 70% (similar to non-LD population); following treatment this may be reduced to 20-55%. This may be because of difficulties in understanding and abiding cautions and probations. The risk of recidivism is highest during the year immediately following discharge.

Chapter 6: Forensic Problems and Offending - Learning Disabilities. (n.d.). Retrieved from <http://hebw.cf.ac.uk/learningdisabilities/chapter6.htm>

Other disorders:

- ★ **Mood disorders:** It is unclear whether bipolar disorder is associated with violence. If it is, then the risk is much lower in magnitude than schizophrenia (i.e. 1/10th of the risk seen in schizophrenia).
- ★ **Depression** is seen as a risk factor for violence, only in the context of other risk factors (psychosis and substance use, for example). It is clearly a risk factor for domestic killings. In particular, depression can act as a disinhibitor after provocations of some form. Psychotic depression can result in nihilism, with a very negative view of the world and, therefore, “mercy killings” within a domestic/family setting.
- ★ **Anxiety disorders:** Between 10-25% of prisoners have anxiety disorders in various forms. There is little evidence of a link between these disorders and offending behaviour. However, severe anxiety can result in dissociation and disinhibition resulting in problem behaviours.
- ★ **Dissociative and conversion disorders:** Depersonalization and derealization can occur in non-mentally disordered individuals at times of stress e.g. Soldiers. Such difficulties can occur for violent offenders at the time of arousal/violence or in response to extreme fear, which may precede violence. A large proportion has been subject to violence in their early life, often seen as a predisposing factor for the development of a dissociative disorder.
 - Violent behaving that is ‘out of character’ may be suggestive of dissociation. However, further evidence is required before relating it to violence. Trait dissociation could be associated with future violence.
 - Dissociate or psychogenic amnesia is associated with violence to a known victim and with the perpetrator often aroused or intoxicated with alcohol. It is very difficult to give clear legal evidence regarding such amnesia in court cases.
- ★ **Epilepsy:** This is rarely linked directly with offending, although this can occur as a result of automatic behaviour during a seizure (ictal) or before (pre-ictal) or after a seizure (post-ictal).
 - About 1% of the population have a diagnosis of epilepsy; the prevalence in prison is slightly more at 1-2%. There is no general increase in violent acts with epilepsy. Ictal violence is rare but more likely in partial vs. tonic-clonic seizures.
 - There are possibilities of two related conditions (referenced in the literature):
 - Episodic dyscontrol syndrome – lack of memory for explosive violence
 - Explosive personality or intermittent explosive disorder (recognised by DSM)
- ★ **Asperger’s syndrome:** Frequent overlap with schizoid PD – some studies show autistic spectrum problems commit more violent offences and criminal damage than the general population.
 - Theory of mind deficits, the presence of egocentricity and deficits in social reciprocity can contribute to the lack of awareness of the impact on victims and of what is wrong in social and emotional terms.

- These factors are especially relevant for sexual offending though there is no particular increase in violent sexual offences among those with Asperger's syndrome.
- ★ **Sleep disorders (parasomnias):** This is a common component of psychiatric disorders, but distinct from sleep walking / other parasomnias e.g. Sleep terrors and nightmares. These disorders are commoner in children.
 - When violence occurs in the context of sleep disturbance –it is commoner in males
 - Usually, there is a previous history of parasomnias including nocturnal enuresis
 - Sleep deprivation/ cannabis/ alcohol and caffeine – can be precursors to sleep walking
 - The sleepwalking offender usually knows his or her victims. The sleepwalker appears to not hear cries or recognise victim.

3. Special syndromes relevant to forensic psychiatry

- ★ **Malingering:** Malingering involves intentional feigning of symptoms of illness for a specific gain. Some suggest using prolonged admission for assessment, use of various instruments SIRS, SIMS (structured inventory of reported/malingering symptoms), and the TOMM (test of malingering memory) for an assessment. Polygraphs are used in the US but rarely in the UK.
- ★ **Munchausen syndrome:** It is a factitious illness where symptoms are created intentionally, motivated to assume the sick role with the absence of external incentives. Usually the type of symptom (that is, psychological or physical) is specified.
 - If accompanied by a factitious illness in a child or dependent it is known as “Munchausen by proxy.” It is associated with personality disorders and the individual often displays attention seeking behaviours, masochistic tendencies and medication seeking behaviours.
 - Management is through safeguarding processes, treating any depressive disorder and efforts to limit behaviours (for example, keeping a hospital register of such patients).
- ★ **Morbid jealousy (Othello syndrome):** This is a syndrome associated with a number of different mental disorders. The key feature is that there is a core belief of the partner’s infidelity. The commonest conditions include paranoid personality disorder or from an underlying psychotic condition.
 - These beliefs are often associated with confirmatory behaviours such as stalking or repeated phone calls. There are often several associated risks: risk of partners, children, associated self-harm and substance misuse. Often, the concerns regarding domestic violence or homicide are significant.
- ★ **Erotomania (De Clerambault’s syndrome):** This is commoner in females than males and is the belief that someone (often more successful than themselves) is in love with them. This is associated with psychotic disorders. Mullen (2008) recognises this as related to stalking or evolving into pathological jealousy, with violence occurring as a result of rejection towards the perpetrator.
- ★ **Ganser syndrome:** Ganser’s syndrome, considered as a hysterical dissociative disorder includes approximate answers, clouding of consciousness with disorientation, psychogenic physical symptoms – analgesia & hyperaesthesia and pseudohallucinations – though the full spectrum of symptoms are not always present. Patients with Ganser’s syndrome are amnesic for their abnormal behaviour; it is often seen in prisoners (esp. military prisoners) and individuals absenting themselves from the army or navy.

4. Special groups of mentally disorder offenders

Female offenders

Various differences have been reported between male and female offenders. The major differences that are important for revision include the following:

- ★ Disproportionately fewer women offend: In a cohort followed up from 1953 birth, 1/3rd of men and 9% of women got convicted at least once by age 46. Among these, 8% of men were responsible for 2/3rd of all crimes. Only 1 in 5 offenders is a woman.
- ★ But women have their peak offending age 2-3 years earlier than men. The peak age of offending is 14 in girls; 17-18 in boys.
- ★ The sex ratio of convicted men to women is approximately 5:1. The National Confidential Inquiry into Suicide and Homicide in 2003 reported that 55% of homicide victims were young men and in only 3% of homicides in UK, both the perpetrator and the victim were female.
- ★ The index offence in women is more likely to be 'romantic' or domestic disputes. 68% of women are in prison for non-violent offences (compared with 47% for men)
- ★ Victims of women's violent acts are more likely to be family members
- ★ When involved in fraud or property offences, women steal fewer items and items of lesser value than men
- ★ A quarter of all women in prison had been sentenced for drugs offences.
- ★ For first time violent offence, both men and women stand similar chance of conviction, but with repeated violence, women have lesser chance of receiving a custodial sentence
- ★ Women make more 'psychiatric' pleas than men; women receive more psychiatric disposals than men
- ★ Over half of women in prison are estimated to be mothers
- ★ Reoffending rates for women discharged from prison, though slightly lower than those for men, remain relatively high: 46.6 for men and 45.0 for women (2006 cohort). However, when they do re-offend, females discharged from prison are more prolific offenders than men: 445.0 offences per 100 male reoffenders compared to 485.8 per 100 female re-offenders.
- ★ Women in custody are five times more likely to have mental health problems than women in the general population.
- ★ Up to 80% of women in prison have diagnosable mental health problems and one in five women on remand in 2003 were identified as suffering from psychosis.
- ★ Her Majesty's Inspectorate of Prisons (HMIP) reported that in 2006/07 a fifth (19%) of women had attempted suicide during the year before custody, nearly three times the rate reported for men. The number of incidents reported for women is also four times higher than that of men.
- ★ A third of women offenders (33%) presented an accommodation need.
- ★ 31% of female offenders who were assessed had an alcohol misuse problem (vs. 42% men)

- ★ “60% of women had problems recorded against emotional well-being (a proxy for psychological and psychiatric issues) compared with 36% of men. In many cases, there are high levels of mental health issues and social isolation, including self-harm, and low-self esteem as a result of a history of victimisation” (UK Cabinet Office, 2014).

Young offenders

- ★ In England & Wales, the age of criminal responsibility is 10 years (in Scotland, 8 years). Young offenders aged 10 to 17 years old are classified as a juvenile offender. Between the ages of 18 and 21 years old they are classed as young offenders. Offenders aged 21 years and over were known as adult offenders. Young offenders are often dealt with by the youth offending service.
- ★ There are various options available to detain young people on remand, at conviction, and under the civil child care law when unmanageable at home or in foster care.
- ★ These include: Young offenders institutes (YOI) for young offenders (18-21 years old) and juvenile offenders (15-17 years old); Secure training centres and detention schools up to the age of 17 years; Secure children’s homes for young offenders between the age of 12-16 years; Adolescent secure hospitals.

Ethnic minorities

- ★ Those not defining themselves as white British include 15% of the UK population, but they are overrepresented in prison and in secure settings.
- ★ There have been concerns about BME (black and ethnic minority groups) groups within mental healthcare because:
 - More patients are likely to be diagnosed with schizophrenia
 - More patients are admitted involuntarily
 - More are secluded and restrained in hospital
 - More are given custodial sentences and have an over-representation of the criminal justice system
- ★ Some individuals from some ethnic groups can face isolation and estrangement from their families if involved in the criminal justice system as well as face peer discrimination.

Deaf offenders

- ★ This group has a higher prevalence of mental disorder and struggle within society and services due to communication difficulties. There are three main general psychiatric hospitals in the UK, one high secure deaf service and several independent hospitals to provide inpatient care. Such a disability may have an impact on the individuals’ fitness to plead.

5. Effect of victimisation on mental health

- ★ Crime victimisation impacts on an individual's ability to perform social roles: especially occupational performance, roles related to parenting, intimate relationships, and affects general social functioning.
- ★ Studies looking predominantly at non-violent crimes indicate that being victims of such crimes have relatively little impact on people's satisfaction with the quality of their lives or happiness. In general, the more violent a crime, the more serious the social dysfunction seen in victims. Victims of sexual assault are particularly prone to experience social withdrawal (Golding et al., 2002). In some studies, >50% of rape victims report a continuously restricted social life even after 15 to 30 months of the assault.
- ★ **PTSD (post traumatic stress disorder):** PTSD is seen more frequently in the forensic population. There is no definite causal link between offending and PTSD in perpetrators. But both perpetrators and victims carry a high risk of PTSD though the rates are clearly higher among the victims. Among prisoners nearly 20% have diagnosable PTSD.
 - Complex PTSD is associated with prolonged abuse often suffered as children. Co-morbid substance misuse and other mental disorders are commonly seen. Psychosis combined with PTSD result in more positive symptoms thus a possible increase in paranoid/violent thoughts/feelings and problem behaviours.
 - The (committed) index offence can be a significant traumatic trigger for forensic inpatients in up to 70% of cases especially if the index offence was unplanned, offender is disinhibited and when there is no prior past forensic history.
- ★ **Patients as victims:** Presence of severe mental illness itself is a risk of being victimised, which further increases psychiatric morbidity. In National Crime Survey (Khalifeh et al., 2015), 40% of patients with a severe mental illness were noted to be victims of crime in the past year, compared to 14% of general population (odds ratio = 2.8); the risk ratio was much higher when violent assaults were considered (19% patients *v.* 3% of controls (OR = 5.3)). Women with severe mental illness were 4-times more likely to be victims of domestic violence; 4-times more likely to be victims of sexual violence and 10-times more likely to be victims of any violence in the community.

Notes prepared using excerpts from

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